

V.5.3.2. Primary forms of infiltrating SCC

Any attempt to derive therapeutic guidelines from the literature concerning squamous cell carcinoma (SCC) faces two major limitations:

- the low number of controlled clinical trials;
- the lack of precise prognostic data in many published studies.

The following recommendations are based on professional consensus and stratification using the OGE scale, which classifies elderly patients according to fragility criteria into three groups:

- **Group 1:** Patients ageing well — fully independent, with minimal impact of ageing on functional capacity. These individuals have higher life expectancy than their age peers and are likely to benefit from cancer treatments similarly to younger adults.
- **Group 2:** Vulnerable patients — intermediate group requiring comprehensive geriatric assessment to optimize and individualize cancer therapy.
- **Group 3:** Fragile patients — reduced life expectancy, at high risk for treatment complications due to organ dysfunction; management should prioritize comfort and short-term quality of life.

For all elderly cancer patients, treatment decisions must be guided by a thorough evaluation of somatic, psychological, and social status, including assessment of frailty.

Treatment Recommendations by Risk Group

Low-Risk SCC (Group 1): Low risk of recurrence or metastasis

- **Preoperative information:** Patients must be informed during a preoperative consultation that meets legal and ethical standards for informed consent.
- **Standard treatment:** Surgical excision with lateral margins of **4–6 mm** from clinical borders and deep margin into subcutaneous tissue, followed by histopathological examination of oriented specimens.
- **Diagnostic uncertainty:** If diagnosis is uncertain, perform an initial punch biopsy (≥ 4 mm) for confirmation. Definitive excision can then proceed with appropriate margins.
- **Positive margins (incomplete excision):** Further intervention is required — either repeat standard excision with 5 mm margins or micrographic surgery.
- **Upstaging to Group 2 histology:** If postoperative histology indicates higher-risk features, discussion with an **IDC (Multidisciplinary Cancer Board)** is recommended to reassess management.

High-Risk SCC (Group 2): Significant risk of recurrence or metastasis

- **Preoperative biopsy:** Perform a punch biopsy (≥ 4 mm) to confirm diagnosis before definitive treatment.
- **Standard treatment:** Surgical excision with lateral margins ≥ 6 mm (up to ≥ 10 mm depending on location and risk factors) and deep margin beyond tumor infiltration.
- **Reconstruction planning:** When flap closure or complex reconstruction is planned, ensure complete tumor removal is histologically confirmed — either via intraoperative (frozen section) or delayed analysis (two-stage procedure), using standard or micrographic techniques.
- **IDC involvement:**
 - Preoperatively: to determine optimal surgical and adjuvant strategy.

- Postoperatively: to validate completeness of treatment and need for additional therapy.
- **Poor prognosis cases:** IDC discussion should guide decisions on:
 - Margin width
 - Method of margin control (standard vs. micrographic)
 - Reconstruction type
 - Need for adjuvant treatment (e.g., radiotherapy)

Non-Surgical Candidates

When surgery is contraindicated (due to medical comorbidities or patient refusal) or would result in significant functional or cosmetic impairment (e.g., lesions in periorificial areas, nasal groove, retroauricular region):

- Full patient information on risks and benefits of alternatives is mandatory.
- Therapeutic options include:
 - **External beam radiotherapy** (electron therapy)
 - **Interstitial brachytherapy** with iridium-192 — particularly suitable for anatomically complex sites
 - **Combined radiotherapy and chemotherapy**
 - **Thermochemotherapy** of a single limb — as neoadjuvant or palliative approach

All such decisions should be made within the framework of **IDC discussion**.

Post-Therapeutic Monitoring

Refer to section **V.5.4** for follow-up recommendations.